

HUMAN GROWTH AND DEVELOPMENT MCQ'S

1. A 9-month-old infant at a well-child visit has had minimal weight gain since 6 months, decreased head growth percentile, and poor feeding. Which immediate nursing action is most appropriate?
 - A. Reassure the mother that some infants grow slower.
 - B. Schedule routine follow-up in 3 months.
 - C. Assess feeding practices, measure weight/head circumference, and refer for growth evaluation.
 - D. Start iron supplementation immediately.

Answer: C

Rationale: Clinical growth monitoring requires measurement and assessment of feeding/environmental causes and referral for early intervention when growth falters. (Physical growth, tracking weight/head circumference).

2. A 4-year-old not yet speaking in two-word phrases — which is the best nursing interpretation?
 - A. Normal; continue routine monitoring.
 - B. Suggest immediate speech therapy referral without further assessment.
 - C. Screen for hearing loss, screen developmentally, and counsel parents on stimulation.
 - D. Diagnose autism spectrum disorder.

Answer: C

Rationale: Developmental delays warrant screening (including hearing) and targeted interventions; clinician should assess before specialist referral. (Developmental milestones & nursing role).

3. A pregnant woman asks why genetic counseling is recommended after a family history of autosomal dominant disease. The nurse's best explanation is:
 - A. "You should accept the disease; nothing can be done."
 - B. "Genetic counseling can estimate recurrence risk and discuss testing options."
 - C. "Only environmental factors cause disease — genetics are irrelevant."
 - D. "You should avoid having children."

Answer: B

Rationale: Nurses should explain that counseling helps parents understand inheritance patterns, testing, and management options. (Genetic foundation; nurses' role).

4. A 15-year-old male presents with delayed secondary sexual characteristics (no voice change, no facial hair). The nurse should first:
 - A. Counsel about social pressures and ignore physical exam.
 - B. Obtain a detailed growth and puberty history and refer for endocrine evaluation.
 - C. Prescribe testosterone immediately.
 - D. Diagnose constitutional delay without assessment.

Answer: B

Rationale: Delayed puberty needs history, growth chart review and appropriate referral—don't treat empirically. (Maturation, sexual maturation).

5. In assessing a newborn, the nurse notes a single palmar crease and hypotonia. The priority is to:
 - A. Document and arrange genetic testing and pediatric evaluation.
 - B. Assume normal variant and discharge.
 - C. Begin physical therapy tomorrow without pediatric consult.

D. Tell parents it is due to delivery technique.

Answer: A

Rationale: Findings suggest possible chromosomal anomaly (e.g., Down syndrome); prompt evaluation and testing are needed. (Chromosome abnormalities; Down syndrome).

6. A mother in a low-SES community reports working long hours and limited time for her toddler. Which statement by the nurse is most appropriate?

A. "Professional success is more important than parenting."

B. "We can discuss strategies to increase stimulation and safe caregiving resources."

C. "You should quit your job immediately."

D. "SES has no effect on development."

Answer: B

Rationale: Nurses should offer practical support/strategies and resources recognizing SES impacts family functioning and child development. (SES and family functioning).

7. A 2-year-old with regression of language and social skills after a fever and hospital stay — nurse notes this as:

A. Typical variability; ignore.

B. Nonnormative life event possibly precipitating developmental regression; arrange assessment.

C. Clearly autism and start ABA therapy.

D. Due to poor parenting only.

Answer: B

Rationale: Nonnormative influences (illness/hospitalization) can disrupt development; assess and intervene. (Nonnormative influences).

8. A clinic is creating a program to support caregivers in a collectivist community. A culturally sensitive nursing approach is to:

A. Encourage only nuclear-family involvement.

B. Exclude extended family from education sessions.

C. Include extended family members and respect local caregiving norms.

D. Insist on Western parenting models.

Answer: C

Rationale: Cultural context matters; in collectivist societies include broader family to be effective. (Cultural context; collectivist vs individualist).

9. Which of the following best exemplifies plasticity in development relevant to nursing care?

A. A child with early brain injury showing improved language after early therapy.

B. A child's IQ is fixed and unchangeable.

C. Developmental milestones never change.

D. Adults cannot learn new skills.

Answer: A

Rationale: Plasticity describes capacity to change with experience/intervention—supports early therapy to improve outcomes. (Plasticity).

10. A 3-month-old's head circumference falls from 50th to 5th percentile in 2 months. The nurse's immediate step:

A. Reassure family—this is fine.

B. Reweigh the infant only.

C. Re-measure, review feeding, and refer for neurodevelopmental and medical evaluation.

D. Begin formula feeding without assessment.

Answer: C

Rationale: Rapid decline in head growth is concerning; requires confirmation and prompt evaluation for underlying causes. (Physical growth monitoring).

11. A nurse counsels parents about critical periods. Which advice is correct?

- A. "All skills develop anytime; timing doesn't matter."
- B. "Some abilities (e.g., language) have sensitive periods when stimulation is vital."
- C. "Critical periods are myths."
- D. "Skills develop only through genetics."

Answer: B

Rationale: Certain developmental windows (sensitive/critical periods) need appropriate input for optimal development. (Critical periods of development).

12. A newborn screening shows a metabolic disorder. The nurse should:

- A. Explain the condition, begin appropriate referrals, and support parents emotionally.
- B. Tell them the screening is always wrong.
- C. Tell parents nothing until the doctor explains.
- D. Recommend stopping breastfeeding immediately without guidance.

Answer: A

Rationale: Nurses play role in guiding parents through diagnosis, referrals, and emotional support. (Nurses' role; detection methods).

13. In counseling about nature vs. nurture, which nursing statement is most accurate?

- A. "Only genes shape the child's future."
- B. "Only environment matters; genes don't."
- C. "Both genetic and environmental factors interact to shape development."
- D. "Neither have real influence."

Answer: C

Rationale: Development results from interaction between genotype and environment. (Nature and nurture discussion).

14. A 7-year-old with ADHD from a family with high parental conflict shows worsening behavior. Nursing intervention that addresses indirect family influence is:

- A. Focus solely on medication.
- B. Refer family for counseling and provide parenting strategies to reduce conflict exposure.
- C. Remove child from school.
- D. Tell parents nothing can be done.

Answer: B

Rationale: Family dynamics indirectly affect child behavior; addressing marital conflict and parenting improves outcomes. (Indirect family influence).

15. A health educator plans a workshop on developmental milestones for caregivers in a rural village. Best plan is to:

- A. Use only text-heavy handouts.
- B. Use demonstrations, local language, and involve community elders.
- C. Offer online webinars only.
- D. Exclude fathers.

Answer: B

Rationale: Tailoring to local context (language, demonstrations, elders) increases reach and cultural relevance. (Environmental/cultural context).

16. A mother worried that her 10-year-old spends hours gaming and has declining school performance. The nurse's best response:

- A. "This is solely a parenting failure."
- B. "Assess routines, screen for psychosocial issues, and discuss balanced activities"

and limits.”

C. “Immediate referral for institutionalization.”

D. “Ignore it; children always outgrow it.”

Answer: B

Rationale: Assess contributing factors (family, school, mental health) and counsel on structured routines; interventions should be individualized. (Multidimensional influences).

17. A newborn with ambiguous genitalia is born. The nurse should first:

A. Make assumptions about gender based on genital appearance.

B. Provide neutral, supportive information and facilitate specialist (endocrine/genetics) evaluation.

C. Assign gender and discharge.

D. Delay communication with parents.

Answer: B

Rationale: Sensitive, supportive communication and timely specialist referral are essential for ambiguous genitalia. (Sex chromosomes; nurses’ role).

18. In discussing cohort effects, the nurse explains:

A. “People born the same period may share traits due to shared historical events.”

B. “Cohorts are irrelevant to development.”

C. “Only genetics cause cohort similarities.”

D. “Cohort effects only influence physical growth.”

Answer: A

Rationale: History-graded influences (e.g., epidemics, tech advances) shape cohorts similarly. (History-graded influences).

19. A 6-month-old being introduced to solids chokes frequently. Nursing priority:

A. Recommend thinner purees only.

B. Assess readiness for solids (head control, sitting), counsel safe feeding techniques, and refer if needed.

C. Start solids earlier to improve swallowing.

D. Ignore—this is expected.

Answer: B

Rationale: Feeding readiness and safety must be assessed; improper timing increases choking risk. (Age-graded influences; teething/solid introduction).

20. A teen girl from an affluent family reports anxiety and pressure to achieve. Nurse recognizes this as:

A. An expected luxury problem with no clinical relevance.

B. A possible psychosocial risk linked to certain high-SES parenting that prioritizes achievement.

C. Not related to family context.

D. Solely genetic.

Answer: B

Rationale: Affluent parenting patterns can cause emotional problems; assessment and support are needed. (SES and family functioning).

21. A newborn with positive family history of X-linked disorder — nurse explanation:

A. “Only boys are ever affected.”

B. “X-linked inheritance can affect males predominantly; genetic counseling and testing are advised.”

C. “It won’t affect this child.”

D. “Environment will prevent it.”

Answer: B

Rationale: X-linked disorders have sex-specific patterns; counseling/testing clarify risk. (Sex-related genetic inheritance).

22. A parent reports toddler frequently misses milestones after a move to a new city. Nurse's best interpretation:

- A. "Moves never affect children."
- B. Consider nonnormative/stressful environmental changes and provide support to ease adaptation.
- C. "Child is lazy."
- D. "Immediate psychiatric admission."

Answer: B

Rationale: Moves are environmental stressors that can temporarily affect development; supportive interventions help. (Family changes; adaptation).

23. A nurse teaching about mitosis and meiosis explains:

- A. "Mitosis produces gametes."
- B. "Meiosis halves chromosome number to form gametes; mitosis duplicates for growth/repair."
- C. "Both processes are identical."
- D. "Only mitosis occurs after conception."

Answer: B

Rationale: Mitosis duplicates cells for growth/repair; meiosis produces gametes with half the chromosomes. (Mitosis vs meiosis).

24. A toddler with recurrent ear infections and language delay — nursing action:

- A. Ignore family concerns.
- B. Screen for hearing impairment and refer to audiology & early intervention.
- C. Start antibiotics indefinitely.
- D. Assume delay is behavioral only.

Answer: B

Rationale: Recurrent otitis media can impair hearing and language development; screen and refer accordingly. (Development; hearing impact).

25. A community outbreak (history-graded influence) disrupts schooling for months. Long-term nursing program should focus on:

- A. Only immunizations.
- B. Supporting catch-up learning, psychosocial support, and family resources.
- C. Ignoring educational impact.
- D. Punishing families for poor attendance.

Answer: B

Rationale: History-graded events require multi-pronged responses including psychosocial /educational interventions. (History-graded influences).

26. A newborn with an extra chromosome (trisomy 21) — nurse counseling includes:

- A. Telling parents nothing can help.
- B. Discussing expected medical issues, early intervention services, and support resources.
- C. Recommending isolation.
- D. Minimizing findings as unimportant.

Answer: B

Rationale: Provide information on associated conditions, interventions, and support—nurse role includes guidance and referral. (Chromosome abnormalities; Down syndrome).

27. In planning anticipatory guidance for parents, the nurse should prioritize:

- A. Information irrelevant to the child's age.

- B. Age-appropriate milestones, safety, feeding, and stimulation strategies.
- C. Only medical facts without practical advice.
- D. Telling parents to find advice online only.

Answer: B

Rationale: Anticipatory guidance tailored to age helps parents support growth and development. (Nurses' role; developmental tasks).

28. A mother asks whether parental education level affects child development. Best reply:

- A. "Education has no impact."
- B. "Parent education is one component of SES that influences resources and child-rearing practices."
- C. "Only income matters."
- D. "Education harms children."

Answer: B

Rationale: Education contributes to SES and influences parenting practices and developmental outcomes. (SES components).

29. A 2-year-old demonstrates normalized repetitive play and toilet training regression after divorce. Nurse intervention:

- A. Blame the child.
- B. Explain regression can occur with stress, provide supportive parenting strategies and referrals as needed.
- C. Ignore because it's temporary.
- D. Recommend immediate medication.

Answer: B

Rationale: Regression after family stress is common; support and guidance help adaptation. (Family changes; adaptation).

30. A newborn female child of sister with fragile X asks about carrier risk. The nurse should:

- A. Provide inaccurate assurances.
- B. Recommend genetic counseling and offer testing information.
- C. Say testing is unnecessary for females.
- D. Recommend pregnancy termination immediately.

Answer: B

Rationale: For X-linked disorders with family history, counseling and testing are appropriate to determine carrier status and inform decisions. (Genetic inheritance; nurses' role).

31. A 14-year-old boy with sudden onset aggressive behavior after losing his job as household breadwinner (older sibling) is likely experiencing:

- A. Only biological disorder.
- B. Emotional/role stress due to changing family responsibilities (non-normative stressor); assess and provide family support.
- C. No change in development.
- D. Purely genetic aggression.

Answer: B

Rationale: Non-normative role changes can lead to psychosocial problems; nursing should assess family context and provide support. (Nonnormative influences; family role).

32. A public health nurse wants to design child development policy interventions. Which principle from the lifespan perspective should guide planning?

- A. Focus only on infancy; other stages irrelevant.

- B. Consider development as lifelong and multidimensional—plan across ages and domains.
- C. Only intervene in adulthood.
- D. Assume one-size-fits-all for all individuals.

Answer: B

Rationale: Lifespan perspective emphasizes lifelong, multidimensional development; policies should address multiple stages/domains. (Lifespan perspective).

33. A mother asks why siblings in same household develop differently. The nurse explains:

- A. “All siblings must be the same.”
- B. “Individual differences arise from genotype, birth order, differential parenting, and unique experiences.”
- C. “Only birth order matters.”
- D. “It’s random and not explainable.”

Answer: B

Rationale: Multiple interacting forces (genetic and environmental) create individual differences even within the same family. (Multidimensional influences).

34. A newborn from a mother who used isotretinoin during pregnancy — nurse should:

- A. Ignore medication exposure.
- B. Urgently refer for teratogenic risk assessment and specialist follow-up.
- C. Reassure that no effects will happen.
- D. Dismiss concerns as anxiety.

Answer: B

Rationale: Known teratogens require prompt evaluation and counseling about possible congenital anomalies. (Nurses’ role in preconception/pregnancy history).

35. A 5-year-old showing signs of social withdrawal after prolonged parental unemployment — nursing approach:

- A. Provide family-centered support, connect to social services, and monitor the child’s psychosocial status.
- B. Punish the child for withdrawal.
- C. Tell the family economic issues don’t affect children.
- D. Refer to juvenile detention.

Answer: A

Rationale: Economic hardship affects family functioning and child wellbeing; nurse should connect families to supports. (SES; family functioning).

36. Which statement best reflects the nurse’s role in genetic-risk situations?

- A. Nurses should never discuss genetics.
- B. Nurses provide initial education, gather family history, and refer to genetics specialists as needed.
- C. Nurses should provide definitive genetic diagnoses.
- D. Nurses should discourage genetic testing.

Answer: B

Rationale: Nurses act as educators and coordinators for genetic risk assessment and referral. (Nurses’ role; genetic foundation).

37. A primary care nurse notices that a 3-year-old’s fine motor skills lag behind peers. Appropriate action:

- A. Ignore until school age.
- B. Screen developmentally, discuss home stimulation activities (drawing, stacking),

and refer occupational therapy if indicated.

C. Advise stopping all play.

D. Label child as unteachable.

Answer: B

Rationale: Early screening and intervention for motor delays (fine motor activities, OT) improve outcomes. (Development; milestones; nurse interventions).

38. A family wants to try indigenous postpartum practices that may restrict maternal nutrition. Nurse response:

A. Insist they stop cultural practices.

B. Respect culture, assess for risk, and provide safe alternatives or supplements as needed.

C. Ignore possible nutritional risks.

D. Call child protective services immediately.

Answer: B

Rationale: Respectful cultural competence involves balancing cultural practices with health promotion and safe alternatives. (Cultural context; nurse role).

39. A toddler exposed to lead in an older home has decreased playfulness and poor attention. Priority nursing actions include:

A. Advise nothing can be done.

B. Arrange lead screening, educate family about exposure reduction, and coordinate medical follow-up.

C. Blame parents for poverty.

D. Wait a year to reassess.

Answer: B

Rationale: Environmental toxins (history/nonnormative influences) require prompt screening, mitigation, and referral. (Environmental context; history-graded/nonnormative hazards).

40. During counseling, a nurse explains why early childhood stimulation programs are effective:

A. "They don't have measurable effects."

B. "They take advantage of sensitive periods and plasticity to improve long-term cognitive and social outcomes."

C. "Only genetics determine outcomes."

D. "Programs only matter in high-SES families."

Answer: B

Rationale: Early stimulation targets sensitive periods and leverages plasticity to yield lasting benefits across domains. (Plasticity; critical periods; lifespan perspective).